INVESTOR BRIEF · CONFIDENTIAL · APRIL 2026

Larinova

1.1 million Indian doctors. One English-only AI scribe.
One alternative — speaking all 22 Indian languages.

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22 / 0
Indian languages we ship vs. competitors at this depth

₹999
Per doctor per month · ~10% under Eka.Care · 85% gross margin

Live
5 alpha doctors · 2–3 hospitals incoming · Day 7 since incorporation

Prepared for Mr. Sundar Venkatraman, CEO · APAR Innosys LLP, Chennai
By Gabriel Antony Xaviour, Founder & CEO · Larinova Private Limited
The ask ₹75 Lakh for 10% equity at ₹7.5 Cr post-money valuation

THE PROBLEM

Indian doctors are losing 90 minutes a day to a typewriter — and the patient's language to translation.

Every Indian GP sees 30+ patients a day. Every consultation generates a SOAP note, an ICD-10 code, and a prescription that has to be typed into an EMR designed in English. But patients describe symptoms in Tamil, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati — 22 languages in total. The doctor mentally translates as they type, and clinical nuance is lost in that translation.

90 min/day

Typed manually after 30+ consultations — a full hour and a half stolen from patients or from going home

22 languages

Spoken by Indian patients in OPDs every day. Existing AI EMRs support only English and Hindi.

~65K doctors

On Eka.Care today — the only AI scribe in India. English / Hindi only. Scribe-only, no booking, no follow-up.

5 disconnected apps

Most clinics juggle separate tools for booking, intake, EMR, scribe, and follow-up — none of them talk to each other

The Indic-language clinical fidelity gap is not a feature — it is a clinical accuracy problem. Mental translation under time pressure produces worse notes, worse prescriptions, and worse outcomes. No platform in the Indian market today closes that gap across all 22 official languages.

THE SOLUTION

One platform for the entire OPD — booking to follow-up.

Larinova is the only end-to-end OPD platform for Indian doctors. Patients book online, complete an AI-guided intake before they arrive, and the doctor walks in with a 60-second Prep Brief in hand. During the consult, Larinova transcribes any of India's 22 official languages — Tamil, Hindi, Telugu, Bengali, Marathi, Kannada, Malayalam and beyond — and generates SOAP notes, ICD-10 codes, and a signed prescription in real time. After the visit, an agent follows up at days 1, 3, and 7.

01 · BOOK	02 · INTAKE	03 · PREP	04 · CONSULT	05 · FOLLOW-UP
Online booking	**AI-guided form**	**60-sec Prep Brief**	**Indic-native scribe**	**Wellness agent**
Patient picks a slot from a public page. Self-confirms by email.	Patient fills before arrival. AI follows up for missing details. Lab uploads accepted.	Doctor sees red flags, prior meds, intake summary the moment patient walks in.	Real-time SOAP notes, ICD-10 codes, signed prescription — any of 22 Indian languages.	Conversational check-ins on day 1, 3, 7. Escalates to doctor only when needed.

THE MOAT

The only OPD platform that speaks every Indian language — and the cheapest AI medical scribe in the world. ABDM / ABHA native. DPDP Act 2023 compliant. Powered by Sarvam AI's Indic models.

THE PRODUCT

A doctor's full clinic day, in 9 minutes of total admin.

Larinova replaces the 90 minutes of post-consult documentation with a 90-second review-and-sign per patient. The doctor speaks naturally — in any Indian language, English, or mixed. Larinova handles the rest. Live in production at **app.larinova.com** — Sundar can sign up and try it himself.

A TYPICAL CONSULT — WHAT THE DOCTOR EXPERIENCES

9:58 AM	Patient arrives. **Prep Brief** is on doctor's screen — symptoms, prior meds, two flagged red flags.
10:00 AM	Consult begins. Doctor presses **Start Recording**. Larinova listens in the patient's language.
10:11 AM	Consult ends. **SOAP note, ICD-10 code, and prescription** appear on screen — in English, drafted from the spoken consultation in the patient's language.
10:13 AM	Doctor reviews, edits one line, signs. Patient leaves with a **signed Rx via email**. Total admin: **under 2 min**.
+1 DAY	**Wellness agent** checks in with the patient: "How are you feeling today?" Replies are classified — only flagged ones reach the doctor.

> Every line above is live in production today. Larinova is a Progressive Web App — works on every doctor's phone or tablet, no install required. Service worker verified, offline fallback wired.

WHERE WE STAND TODAY

Day 7 since incorporation. Real users on a live product.

Larinova Private Limited was incorporated on 22 April 2026. We are deliberately presenting Day 7 numbers honestly — small, real, and growing fast — rather than inflated metrics that would not survive Sundar's fact check. Here is exactly what is true, as of today.

ACTIVE ALPHA DOCTORS

5 onboarded

Chennai launch cohort, Tamil-speaking. Whitelisted as alpha pro. Co-building with founder weekly.

HOSPITAL PIPELINE

2–3 inbound

Expected to onboard within the next 30 days. Multi-doctor deployments, enterprise wedge.

PRODUCT STATUS

100% live

Booking, AI intake, Prep Brief, Indic scribe, follow-up agent — all in production at app.larinova.com.

PRICING LOCKED

₹999 / mo

Pro tier, billed monthly or ₹9,990/yr. ~10% under Eka.Care for a fuller product.

INDIC CLINICAL AI

22 languages live

All 22 official Indian languages, day one — via Sarvam AI's Indic models. The only such platform in India today.

CAP TABLE

100% founders

Gabriel 90% / Father 10% (governance). Zero outside money. Zero advisor equity. Zero SAFE notes.

WHAT WE COMMIT TO IN THE NEXT 30 DAYS

→ **8–10 active paying doctors** (from 5 today, all on Pro tier conversion after free-trial use)

→ **First hospital pilot signed** — multi-doctor deployment in Chennai

→ **First Indonesian doctor onboarded** as cross-border proof

→ **SH-7 filed** to raise authorised capital — share allotment process readied for Sundar's funds

THE MARKET

The largest underserved healthcare-software market on earth — and we speak every language in it.

The Indian medical-software market is dominated by clinic-management tools (Practo, Clinikk, Healthplix) and a single English / Hindi AI scribe (Eka.Care). Larinova serves all 1.1 million Indian doctors from day one. The largest unserved opportunity sits inside the ~420,000 doctors whose patients speak Tamil, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati, Odia, Assamese — languages no current AI EMR understands.

SEGMENT	DOCTORS	CURRENTLY SERVED BY	LARINOVA FIT
Tamil (TN, Puducherry, parts of KA) — Chennai launch cohort	~140,000	Nobody — English EMRs only	**Live day one. Home advantage. ICP #1 by geography.**
Telugu / Bengali / Marathi (AP, TS, WB, MH)	~280,000	Nobody	**Live day one — same product, native in every Indic language via Sarvam.**
Hindi-speaking (UP, MP, BR, RJ, DL)	~390,000	Eka.Care (scribe only)	**Live day one — competing on full OPD stack + price.**
English-primary (urban metros)	~300,000	Eka.Care, Practo, Healthplix	**Live day one — competing on full stack + multi-language flexibility.**
India total addressable	**~1.1M**	Fragmented — none cover all Indic languages clinically	**Larinova serves all of it from day one.**

At **₹999 / month per doctor**, India's 1.1M-doctor addressable market is **~₹13,000 Cr / year** in revenue ceiling. The 420K underserved Indic-language doctors alone — those with no AI EMR alternative today — represent **~₹500 Cr / year**. Capturing just 5% of that underserved segment is **~₹25 Cr ARR**, without ever competing for a doctor who has another option.

COMPETITION · MOAT

No competitor does what we do — at any price, in any market.

DIMENSION	LARINOVA	EKA.CARE (INDIA)	NUANCE / SUKI / ABRIDGE (US)
Languages	All 22 Indian languages live day-one (Tamil, Hindi, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati, Odia, Assamese, +)	English + Hindi only	English only
Product scope	Full OPD: booking + intake + Prep Brief + scribe + follow-up	Scribe only	Scribe only
Patient booking	Public booking page, self-confirm	None	None
Pre-consult AI	60-sec Prep Brief on every patient	None	None
Wellness follow-up	Conversational agent on day 1, 3, 7	None	None
ABDM / ABHA native	Full	Partial	Not applicable
Price per doctor	₹999 / month	~₹1,100 / month	$208 – $830 / month (₹17K – ₹70K)

THE STRUCTURAL MOAT

Multi-Indic clinical AI is not a feature competitors can flip a switch on — it requires integration with Sarvam's full Indic model family and clinical-domain training across each language. Eka.Care has chosen English/Hindi for three years and shows no roadmap signal to expand. We are the only full-OPD-stack platform in the Indian market — the moat is product breadth + 22-language depth + pricing leadership simultaneously.

BUSINESS MODEL · UNIT ECONOMICS

A SaaS that becomes profitable per-doctor on the first paid month.

Larinova is a per-doctor monthly subscription. ₹999 / month for unlimited consultations on the Pro tier; a free tier capped at 20 consultations per month feeds top-of-funnel and converts to paid as usage scales. Cost to serve is dominated by AI inference (Sarvam STT + Anthropic LLM), which we route through our own Claude Service infrastructure to cap unit cost.

ARPU PER DOCTOR

₹11,988

Annual recurring revenue per Pro doctor at ₹999/mo (or ₹9,990/yr at the discounted annual rate).

GROSS MARGIN

~85%

Cost to serve ~₹150/mo per doctor (Sarvam + Anthropic + Supabase + Vercel infra), all variable.

CAC PAYBACK

< 1 month

₹1,500 ambassador commission per activation + ₹2,500 on paid → recovered in first paid month.

PATH TO ₹1 CR ARR

~840 doctors

At ₹999/mo Pro. Achievable in 14–18 months with the funded GTM motion.

PATH TO ₹10 CR ARR

~8,400 doctors

~0.8% of India's 1.1M doctors. Year 3 target with hospital-tier expansion.

HOSPITAL TIER ACV

₹3 – 12 L

Mid-size hospital (15–60 doctors). One signed deal = 8–25 individual conversions.

Industrial-CEO test: at 85% gross margin, ₹999/mo per seat, sub-1-month CAC payback, Larinova has the unit economics of a category-leading SaaS — even before we layer on hospital-tier deals. Profitability per doctor begins on day 30.

INDONESIA · THE SECOND ENGINE

India is primary. Indonesia is validated optionality.

India is Larinova's first market and remains the focus of this round. But the same Larinova landing page is live in Bahasa Indonesia at **larinova.com/id**, and the opportunity in Indonesia is structurally even cleaner: 180,000+ doctors and zero direct AI scribe competitor. We treat it as the second engine — designed to validate a cross-border thesis without diluting India focus.

Why Indonesia is the cleanest second market in healthcare AI

Indonesian clinic management software (Klinik Pintar, Assist.id, Trustmedis) costs Rp 250K–3M/month and does scheduling and records — but **none auto-generate clinical notes**. There is no Bahasa-native AI scribe in the Indonesian market today. The closest entrant — Medow Health AI from Singapore — has not launched in Indonesia. Global premium scribes (Nuance, Suki) cost $200–$830/month per doctor and have no Bahasa support. Larinova at **IDR 299K/month (~$18)** is structurally uncatchable on price + language.

MARKET SIGNAL	NUMBER
Doctors in Indonesia	~180,000
Direct AI scribe competitors	**0**
Larinova price vs. global premium scribes	**82 – 99% cheaper per doctor**
Larinova price vs. local clinic software (Klinik Pintar etc.)	**~50% cheaper, with capabilities they don't have**
Indonesia funded scope this round	₹8 L of ₹75 L (~11%) — proof, not full launch

12-MONTH ROADMAP · RISKS

Concrete milestones. Honest risks.

MONTH 1–3	**50 paying Chennai doctors** · 3 Chennai clinic deals · first hospital pilot signed · ambassador program live with Bala + 2 founding ambassadors
MONTH 4–6	**120 paying doctors** · 2 hospital deployments · ₹1.4 L MRR · Indonesia first 5–10 paying doctors as cross-border proof
MONTH 7–9	**250 paying doctors** · 5 hospital deals signed · expansion to Coimbatore + Madurai · first strategic hire (BD or engineering)
MONTH 10–12	**500 paying doctors** · ~₹50 L ARR · break-even line of sight · Series Seed conversation initiated at **₹25–50 Cr post-money** — 5–10× markup for Sundar

RISK	MITIGATION
Eka.Care expands beyond English/ Hindi	We are 22 Indic languages live day one vs their 2; deeper ABDM moat + full OPD stack they do not have; Indic-language clinic relationships locked in by then
Solo founder bus factor	This raise funds the first strategic hire by month 9; AI augmentation already de-risks single-person execution
Doctor adoption slow	Co-build weekly with 5 alpha doctors; free trial removes friction; ambassador program creates peer pull
Indonesia regulatory delays	India is primary, Indonesia is upside not load-bearing — round closes regardless of Indonesia trajectory
AI infra costs exceed revenue	~85% gross margin even today; Claude Service caps unit cost; free vs Pro tier controls runaway usage

TEAM · CAP TABLE

A solo, full-time, AI-augmented founder — with a four-year track record of shipping.

Gabriel Antony Xaviour

FOUNDER & CEO · 22 · CHENNAI

- **Bachelor of Engineering, Electronics & Communication** — Loyola-ICAM College of Engineering & Technology (LICET), Chennai
- **70+ hackathon wins in four years** across AI, Web3, and consumer software — top-decile competitive product builder in India
- **Founding Engineer at RPS AI** (San Francisco) — backed by Orange DAO, Delphi Ventures, TRIVE Singapore, UF Venture; raised millions across early rounds
- **Solutions Engineer at Marlin Protocol** (Dubai) — trusted execution environments and privacy compute infrastructure
- **Engineer at Rax Tech** (Chennai) — IoT, software, and e-surveillance; ₹20–40 Cr/year revenue; founded and run by his parents
- **Solo, full-time, AI-augmented** — building Larinova end-to-end, technical and commercial, with Claude as a force multiplier across engineering, sales, and operations

> "The biggest battle in technology right now is healthcare AI. I want to win it from India."

Father (Antony Xaviour, DIN 01646014) — co-founding subscriber on the Memorandum of Association, holds 10% as a governance shareholder. CEO of Rax Tech in his own right; no operational or advisory role in Larinova. **Bala (Balachandar Seeman)** — independent BD partner on commission-only structure; no equity.

HOLDER	ROLE	PRE-ROUND	POST-ROUND
Gabriel Antony Xaviour	Founder & CEO	90.0%	81.0%
Antony Xaviour	Co-founding shareholder (governance only)	10.0%	9.0%
Sundar Venkatraman	Lead angel — new this round	—	10.0%
Total		**100.0%**	**100.0%**

THE ASK

₹75 Lakh for 10% equity. 22 months to a Series Seed at 5–10× markup.

₹75,00,000

For 10% equity · at ₹7.5 Cr post-money valuation

22 months runway · clean equity, no SAFE, no convertible

Funds land into Larinova Pvt Ltd current account post SH-7 capital increase (~10 days)

USE OF FUNDS — ₹75L OVER 22 MONTHS	AMOUNT
Founder runway (₹1L/mo × 22 months)	₹22 L
Hospital BD travel + Bala commissions	₹10 L
First strategic hire (month 9–12)	₹12 L
Indonesia market entry (proof, not full launch)	₹8 L
Marketing + content + brand video production	₹6 L
AI inference + infra (Sarvam, Anthropic, Supabase, Vercel)	₹6 L
Compliance + legal (GST, DPDP, HPR, MSME, ToS)	₹4 L
Tools + subscriptions (Claude, Cursor, Linear, etc.)	₹3 L
Buffer for unknowns	₹4 L
Total	**₹75 L**

What this commits us to: 500 paying doctors, ~₹50 L ARR, line of sight to break-even, Series Seed conversation at ₹25–50 Cr post-money — by Month 12.

Why Sundar: the cheque comes from a relationship that already exists — APAR Innosys has trusted Rax Tech for years. Larinova is the next chapter from the same family, with measurably better unit economics and a 50× larger market. Sundar gets first-mover position before Larinova's Series Seed.

Next step: Sundar's review & counter. We are ready to sign within 7 days of agreed terms.